

Mental Health Taboos, Blind Faith & Common Issues in India — Compilation

Compiled for: Manas Ranjan Jena | Date: 2025-08-27

Purpose: This document lists taboos and blind faith related to mental health in India; names common and dangerous mental-health-related issues faced by youth and adults; and describes how society commonly perceives these issues. Solutions are intentionally omitted.

This compilation takes into account the previously provided report 'Mental Health in India (2025): Issues, Taboos' and expands the lists of issues and societal perceptions. (Source: user-provided PDF.)

1. Overview & Scope

Scope: Youth (adolescents, college students, early-career adults) and adults across India are considered. Focus areas: cultural taboos, blind faith practices, causes of distress (sexuality, careers, studies, relationships), and a comprehensive catalog of mental-health-related conditions and dangerous issues. No treatment or solutions are provided here.

2. Major Taboos & Forms of Blind Faith Affecting Mental Health

- Therapy and counselling: Viewed as for 'the mad' or 'weak'—help-seeking avoided.
- Psychiatric medication: Misconceptions that meds 'change personality', are 'addictive', or signify permanent illness.
- Suicidal talk: Myth that talking about suicide 'plants the idea'—leads to silence and concealment.
- Faith healing, exorcism, and possession narratives: Illness attributed to 'nazar', spirits or curses; spiritual rituals substitute for medical care.
- Witch-branding and superstition: Accusations leading to violence or ostracism (reported in some rural regions).
- Men showing vulnerability: Stoic masculinity norms punish emotional expression and help-seeking.
- LGBTQIA+ identities: Seen as 'phase' or 'immoral'—identity stigmatized and access to affirming care limited.
- Menstruation and mood: Period-related mood issues mocked, dismissed, or moralized.
- Perinatal struggles: New mothers expected to be 'joyful'—postpartum depression is hidden or blamed on the mother.
- Leaving relationships/divorce: Social shame prevents people from escaping abusive or harmful relationships.
- Live-in relationships & dating: Moral policing and family dishonour narratives override safety and consent conversations.
- Addiction as moral failing: Substance use often criminalized or moralized rather than treated.
- Somatic presentations: Mental distress expressed as physical symptoms and diverted to multiple medical specialties without psychiatric recognition.
- Sleep deprivation & hustle culture: Sleep loss normalized as virtue—mental health consequences ignored.

3. How Taboos & Blind Faith Alter Behaviour and Outcomes

Common downstream effects: delayed help-seeking, trial of non-evidence remedies, family secrecy, criminalization or punishment of vulnerable people, and increased risk of crisis (self-harm, untreated psychosis, prolonged suffering). Intersectional factors (gender norms, caste, class, sexuality, rural/urban divide) compound stigma and access barriers.

- Families preferring religious rituals/exorcism before or instead of medical consultation—delays in effective treatment.
- Young people conceal issues related to sexuality or identity for fear of rejection—leads to isolation and depression.
- Students and families hide suicidal ideation to avoid 'shame'—reduces opportunities for early safety planning.

4. Potential Causes of Distress & Mental-Health Regression in Youth

Listed below are key causes (direct and indirect) of mental health deterioration among youth in India. Each item includes how society commonly frames or reacts to it (negative perceptions, taboos, and misconceptions).

Sexuality, Sexual Orientation & Gender Identity

Distress: internal conflict, fear of rejection, family rejection, concealment, minority stress, suicidality. Societal perception: Often labeled 'immoral', 'phase', or 'Western'; families may pressure for conversion, forced marriages, or silence.

Academic Pressure & Competitive Exams

Distress: chronic anxiety, sleep loss, panic attacks, depressive symptoms, burnout. Societal perception: Failure seen as personal or family shame; mental health often minimized as 'lack of effort'.

Career Uncertainty, Unemployment & Job Insecurity

Distress: low self-worth, anxiety, financial stress, substance use, suicidal ideation. Societal perception: Blame placed on the individual; unemployment seen as laziness or poor planning; stigma around taking 'low-status' jobs.

Romantic Relationships, Breakups & Sexual Coercion

Distress: heartbreak, trauma, PTSD from sexual violence, shame, social exclusion. Societal perception: Victim-blaming, honor-shame framing, pressure to stay silent to 'protect family honour'.

Family Expectations & Intergenerational Conflict

Distress: identity suppression, career choice pressure, arranged vs love marriage conflicts. Societal perception: Dissent framed as disrespect; youth told to 'sacrifice' for family reputation.

Financial Stress & Debt

Distress: chronic worry, sleep problems, substance use, hopelessness. Societal perception: Debt seen as failure; asking for help viewed as shameful.

Social Media, Cyberbullying & Comparison Culture

Distress: low self-esteem, body-image issues, anxiety, sleep disruption. Societal perception: 'It's normal' or dismissed as mere 'attention-seeking'.

Substance Use & Peer Pressure

Distress: addiction, academic and occupational decline, legal trouble. Societal perception: Moral failing, criminalization, or family denial of the problem.

Bullying, Caste-based Discrimination & Identity-based Exclusion

Distress: trauma, depression, PTSD, dropout, self-harm. Societal perception: Often normalized or minimized; victims pressured to 'move on'.

Gender-based Violence, Domestic Abuse & Marital Coercion

Distress: PTSD, depression, suicidality. Societal perception: Victim-blaming, pressure to preserve marriage, silence and stigma.

Sexual Health Issues & STIs

Distress: shame, secrecy, relationship conflict, fear of disclosure. Societal perception: Morality judgments and ostracism; limited open discussion.

Exam-related Harassment, Ragging & Hazing

Distress: PTSD, dropout, substance use, self-harm. Societal perception: 'Ragging builds character' myth or institutional denial.

Sleep Deprivation & Overwork Culture

Distress: cognitive decline, mood disorders, increased accident risk. Societal perception: Glorified as dedication; rest is laziness.

Migration, Displacement & Urban Stressors

Distress: isolation, lack of social support, acculturative stress. Societal perception: Migrant struggles are invisible or blamed on migrants themselves.

Identity Crises & Existential Distress

Distress: anxiety, depression, dissociation. Societal perception: 'Being dramatic' or 'attention-seeking'.

5. Comprehensive Catalogue: Common and Dangerous Mental-Health-Related Issues

The lists below include commonly encountered conditions, risky behaviors, and severe/dangerous issues affecting youth and adults in India. This is a naming catalog—definitions are brief or omitted by user's request.

Mood & Affective Disorders

- Major Depressive Disorder (Depression)
- Persistent Depressive Disorder (Dysthymia)
- Bipolar Affective Disorder
- Seasonal Affective Disorder (where applicable)

Anxiety & Stress-related Disorders

- Generalized Anxiety Disorder (GAD)
- Panic Disorder
- Social Anxiety Disorder
- Specific Phobias
- Adjustment Disorder
- Acute Stress Reaction

Trauma & Stressor-related

- Post-Traumatic Stress Disorder (PTSD)
- Complex PTSD
- Trauma-related Dissociation

Psychotic Disorders & Severe Mental Illness

- Schizophrenia
- Schizoaffective Disorder
- Brief Psychotic Disorder
- Delusional Disorders

Neurodevelopmental & Neurodivergent Conditions

- Autism Spectrum Disorder (ASD)
- Attention Deficit Hyperactivity Disorder (ADHD)
- Learning Disorders
- Intellectual Developmental Disorders

Substance-related & Addictive Disorders

- Alcohol Use Disorder
- Opioid Use Disorder
- Cannabis Use Disorder
- Polysubstance Use
- Behavioral Addictions (Gaming, Internet, Gambling)

Eating & Body Image Disorders

- Anorexia Nervosa
- Bulimia Nervosa
- Binge Eating Disorder
- Body Dysmorphic Disorder

Mood/Perinatal & Reproductive-related

- Postpartum Depression
- Perinatal Anxiety
- Premenstrual Dysphoric Disorder (PMDD)

Personality & Identity-related

- Personality Disorders (Borderline, Antisocial, Narcissistic, etc.)
- Identity Distress related to sexual/gender identity

Self-harm, Suicidality & Crisis Behaviors

- Non-suicidal Self-Injury (NSSI)
- Suicidal Ideation
- Suicide Attempts
- Completed Suicide (died by suicide)

Somatic & Medically-Related Presentations

- Somatic Symptom Disorder
- Conversion Disorder (Functional Neurological Symptom Disorder)
- Health Anxiety (Hypochondriasis)

Other High-Risk Situations & Social Phenomena

- Ragging/hazing-related trauma
- Witch-branding and mob-violence related trauma
- Honor-related violence and forced marriages
- Severe workplace burnout with suicidal risk
- Internet-facilitated harassment and doxxing

6. Taboos & Misconceptions — Detailed Map

'Therapy is for mad people' — Labeling and fear of being 'mad' reduces early help-seeking; leads to crisis-level presentations.

'Medications change your personality' — Fear of medicine can push families to unproven remedies; adherence is poor when started.

'Talking about suicide puts the idea in the head' — This prevents safety planning and open support; silence increases risk.

'Mental illness = family shame' — Families hide conditions, avoid documentation and formal care; access to benefits and accommodations reduced.

'Spiritual cure before medical care' — Delays effective treatment and may cause harm; in extreme cases, leads to abuse or torture.

'Men must be stoic' — Suppresses emotional expression; men present late with severe symptoms or substance misuse.

'LGBTQIA+ identity is a phase' — Causes family rejection, forced conversion attempts, and denial of identity-affirming care.

'Young people are dramatic' — Minimizes real distress—prevents early intervention.

7. Dangerous Practices & Risks Associated with Blind Faith/Taboos

- Physical abuse during 'exorcism' or faith-healing rituals; restraint and starvation reported in some cases.
- Witch-branding leading to mob attacks, social ostracism, and fatalities.
- Delay to emergency psychiatric care during psychosis leading to violence or self-harm.
- Use of unregulated 'cures' or herbs leading to medical complications.
- Forced isolation or 'reparative' practices for LGBTQIA+ people (psychological and physical harm).

8. Quick Reference Index (Names Only)

Therapy stigma, Medication myths, Suicide silence, Faith healing, Witch-branding, Honor-shame, Men's stoicism, LGBTQIA+ stigma, Menstruation taboos, Perinatal silence, Academic pressure, Career insecurity, Social media harms, Substance stigma, Ragging trauma, Identity-based violence.

9. Source & Disclaimer

Primary basis: user-provided PDF report 'Mental Health in India (2025): Issues, Taboos' and the user's instructions. This document intentionally lists problems, taboos, and names of mental-health-related conditions without offering solutions. If you or someone is in crisis, seek immediate help (Tele-MANAS 14416 or local emergency services).

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